Access guide — aml-mds-related-rr-tp53-aberrant-hla-pending-x7q2-rerun

How to access each therapy: trial contacts + manufacturer medical-info

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Access guide — aml-mds-related-rr-tp53-aberrant-hla-pending-x7

How a patient or treating team could practically access each intervention in this case's dossier. Trial recruitment contacts and manufacturer medical-information lines are captured for direct outreach. The number in the first column of the summary table is the entry's identifier in the per-intervention sections below — use it to find the deep section quickly. Information ages — each row carries a Verified date; re-screen before relying on a specific contact or trial slot.

Summary

#	Intervention	Modality	Access status	Regulatory	Recommended first action
1	Gemtuzumab ozogamicin (CD33 ADC)	ADC	Standard of care	FDA-approved (Mylotarg); includes CD33-positive relapsed AML monotherapy	Request quantitative CD33 antigen-density flow to support the targeting decision, since qualitative positivity does not establish ADC benefit
2	Pivekimab sunirine (IMGN632, CD123 ADC)	ADC	Off-label use + Clinical trial	FDA-approved (Decnupaz, BPDCN, May 2026); off-label for AML	Call AbbVie medical information at 844-663-3742 to discuss off-label AML use and whether any expanded-access mechanism exists
3	Revumenib (menin inhibitor)	small_molecule	Off-label use + Clinical trial	FDA-approved (Revuforj, KMT2A-translocation acute leukemia and NPM1-mutated AML); off-label for this patient	Recognize the biomarker mismatch first: KMT2A amplification is not the KMT2A rearrangement menin inhibitors target, and the patient is NPM1-wild-type
4	Tagraxofusp (SL-401, IL-3/diphtheria-toxin fusion)	other	Off-label use + Clinical trial	FDA-approved (Elzonris, BPDCN, 2018); off-label for AML	Obtain the quantitative CD123 antigen-density flow result, which predicts benefit and is still pending
5	Ziftomenib (menin inhibitor)	small_molecule	Off-label use	FDA-approved (Komzifti, R/R NPM1-mutated AML, Nov 2025); off-label for this patient	Note the biomarker mismatch: neither an NPM1 mutation nor a KMT2A rearrangement is documented, so menin inhibition lacks a target
6	BSB-1001 (HA-1-directed TCR-T)	other	Clinical trial	investigational, no approval	Resolve the HLA-A*02:01 and HA-1 typing before contacting sites
7	BSB-2002 TCR-T)	other	Clinical trial	investigational, no approval	Do not pursue: the patient is NPM1-wild-type and not in the MRD-positive population this trial enrolls

8	CBX-250 (CG1/HLA-A*02:01 TCR-mimetic T-cell engager)	bispecific_other	Clinical trial	investigational, first-in-human	Confirm HLA-A*02:01 status, the single gating biomarker for this engager
9	CD123-CD33 compound CAR-T (cCAR)	CAR-T	Clinical trial + Not yet accessible	investigational, no approval	Email the sponsor at kevin.pinz@icellgene.com (631-538-6218) to confirm whether the trial is still enrolling and whether any US site exists
10	CAR-T	CAR-T	Clinical trial + Not yet accessible	investigational, no approval	Confirm the Shenzhen site is accepting international patients and understand the travel and manufacturing logistics before workup
11	FH-WT1-E50 (anti-WT1 TCR/CD8ab T cells) + azacitidine	other	Clinical trial	investigational, no approval	Hold as a consolidation option rather than a current-state fit given the 85%-blast burden
12	HA-1-specific CD8+/CD4+ TCR-T (Bleakley/Fred Hutch platform)	other	Clinical trial	investigational, no approval	Retrieve the patient's HLA-A*02:01 allele status and genotype HA-1 in both the patient and the sibling donor before any outreach
13	Iomab-B (131I-apamistamab) anti-CD45 radioimmunotherapy conditioning	radioimmunotherapy	Clinical trial	investigational, no approval (SIERRA phase 3 completed; not FDA-approved)	Email the Actinium go-forward-trial contact (347-814-2268) to ask when NCT07157514 opens and whether a prior sibling allo-HCT is allowed
14	MB-dNPM1-TCR.1 TCR-T)	other	Clinical trial + Not yet accessible	investigational, no approval	Do not pursue unless relapse NGS documents a susceptible NPM1 mutation, which the current panels do not show
15	Multiantigen leukemia-specific donor T cells	other	Clinical trial	investigational, no approval	Confirm WT1 and PRAME expression on the patient's blasts, which drives eligibility here
16	Rezatapopt (PC14586, Y220C-selective p53 reactivator)	small_molecule	Clinical trial	investigational, no approval	Resolve TP53 status without collapsing the discrepancy: retrieve the 2019 variant identity and VAF, add p53 IHC and 17p FISH, and get variant-level NGS to establish whether it is Y220C
17	TSC-100 (donor-derived HA-1-specific TCR-T)	other	Clinical trial	investigational, no approval	Resolve HLA-A*02:01 and HA-1 genotypes for patient and sibling donor first, since the whole arm hinges on the mismatch direction

18	TSC-101 (donor-derived HA-2-specific TCR-T)	other	Clinical trial	investigational, no approval	Genotype both HA-1 and HA-2 in the patient and sibling donor, since the result selects between TSC-100 and TSC-101
19	BPX-701 (PRAME TCR-T with iCasp9 safety switch) + rimiducid	other	Unavailable	investigational; program discontinued	No action; the program is discontinued
20	Eprenetapopt (APR-246, pan-p53 reactivator)	small_molecule	Unavailable	investigational; myeloid development largely halted after negative phase 3	No action recommended; the pan-reactivator track is low-yield and myeloid development has stalled
21	Flotetuzumab (MGD006, CD123xCD3 DART)	bispecific_other	Unavailable	investigational; program deprioritized	No action; the program is deprioritized with no open route
22	NTLA-5001 WT1-directed TCR-T)	other	Unavailable	investigational; program discontinued	No action; the program is discontinued
23	Vibecotamab (XmAb14045, CD123xCD3 bispecific)	bispecific_other	Unavailable	investigational; program discontinued	No action; the program is discontinued
24	WT1-TCRC4 EBV-specific CD8+ Tcm	other	Unavailable	investigational; study completed	Treat as precedent only; the open WT1 TCR-T route is FH-WT1-E50 (NCT07645469)
25	WT1-siTCR gene-transduced lymphocytes (TBI-1301)	other	Unavailable	investigational; early-phase study completed	Prioritise resolving HLA-A*02:01 first; this branch only matters if that types negative

Standard of care (1)

1. Gemtuzumab ozogamicin (CD33 ADC)

Aliases: gemtuzumab ozogamicin, Mylotarg, GO, CMA-676

Modality: ADC · **Regulatory:** FDA-approved (Mylotarg); includes CD33-positive relapsed AML monotherapy · **Guidelines:** NCCN-listed for CD33-positive AML · **Geographic scope:** US; approved and available nationally · **Verified:** 2026-07-24

Gemtuzumab is the one CD33-directed option the patient can get by routine prescription: it is FDA-approved for CD33-positive AML including relapse, matching the confirmed CD33 positivity, so no trial or off-label appeal is needed. The load-bearing caveats are efficacy and safety, not access. Single-agent depth in relapse is modest (about 30%), the adverse-cytogenetics subgroup saw no survival benefit in the Hills meta-analysis, and the VOD/hepatic risk matters before a second transplant.

Next steps

1. Request quantitative CD33 antigen-density flow to support the targeting decision, since qualitative positivity does not establish ADC benefit
2. Weigh the null adverse-cytogenetics subgroup finding against this patient's complex/TP53-aberrant karyotype before committing
3. Assess baseline LFTs and VOD risk given the planned HCT2
4. For dosing or drug questions, call Pfizer medical information at 1-800-505-4426

Trial pathways

NCT	Phase	Status	Eligible	Central contact
—	—	available	yes	—

Manufacturer / sponsor contact

- **Company:** Pfizer (Wyeth)
- **Med info phone:**1-800-505-4426
- **Product info:**https://www.pfizermedical.com/mylotarg
- **Notes:** Pfizer US medical information 1-800-505-4426 (Mon-Fri 9:00-17:00 ET).

Payer / coverage: Approved and routinely reimbursed for CD33-positive AML, so this is on-label prescription rather than an access hurdle. Hepatic/veno-occlusive-disease risk is the clinical constraint, particularly ahead of a second transplant.

Off-label use (4)

2. Pivekimab sunirine (IMGN632, CD123 ADC)

Aliases: pivekimab sunirine, IMGN632, pivekimab, PVEK, Decnupaz

Modality: ADC · **Regulatory:** FDA-approved (Decnupaz, BPDCN, May 2026); off-label for AML · **Geographic scope:** US; approved drug, no currently-enrolling AML trial · **Verified:** 2026-07-24

Pivekimab was FDA-approved for BPDCN as Decnupaz in May 2026, which opens an off-label path for the patient's CD123-positive AML, though off-label reimbursement would be an uphill payer conversation. The AML trials that generated its single-agent activity are now closed to enrollment, so a trial slot is not currently available. The veno-occlusive-disease signal deserves weight given the planned second transplant.

Next steps

1. Call AbbVie medical information at 844-663-3742 to discuss off-label AML use and whether any expanded-access mechanism exists
2. Factor the VOD signal into sequencing, since a second transplant is planned
3. Confirm quantitative CD123 density, which tracks ADC potency in the preclinical work
4. Recheck ClinicalTrials.gov periodically in case a new enrolling AML cohort opens

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT03386513	1/2	active not recruiting	unconfirmed	—
NCT06034470	1	active not recruiting	no	—

Manufacturer / sponsor contact

- **Company:** AbbVie (ImmunoGen)
- **Med info phone:** 844-663-3742
- **Notes:** AbbVie US medical information 844-663-3742 (Mon-Fri 8:00-17:30 CT). Approved as Decnupaz for BPDCN May 2026; AbbVie is the point of contact for off-label and expanded-access inquiries.

Payer / coverage: Newly approved for BPDCN only. Off-label AML use would require prior authorization and almost certainly a payer appeal. Both AML trials are closed to enrollment, so the trial route is not currently open.

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3. Revumenib (menin inhibitor)

Aliases: revumenib, SNDX-5613, Revuforj

Modality: small_molecule · **Regulatory:** FDA-approved (Revuforj, KMT2A-translocation acute leukemia and NPM1-mutated AML); off-label for this patient · **Guidelines:** NCCN-listed for KMT2A-rearranged and NPM1-mutated R/R AML; off-guideline for KMT2A amplification · **Geographic scope:** US; approved drug, trial recruiting but biomarker-mismatched · **Verified:** 2026-07-24

Revumenib is FDA-approved, so off-label prescription is technically a path, but the target is effectively absent: the patient has KMT2A amplification rather than the KMT2A rearrangement (or NPM1 mutation) that menin inhibition depends on, so benefit is unproven and off-label coverage would be hard to secure. The AUGMENT-101 trial is recruiting but gates on the rearrangement the patient does not have. This is landscape completeness more than an actionable route.

Next steps

1. Recognize the biomarker mismatch first: KMT2A amplification is not the KMT2A rearrangement menin inhibitors target, and the patient is NPM1-wild-type
2. Only if relapse NGS documents a KMT2A rearrangement or NPM1 mutation, contact Syndax at clinicaltrials@syndax.com (781-419-1400) for AUGMENT-101 or medinfo@syndax.com for off-label discussion
3. The <40 kg expanded-access program does not apply to this adult patient

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT04065399	1/2	recruiting	no	Syndax Pharmaceuticals; clinicaltrials@syndax.com; 781-419-1400

Manufacturer / sponsor contact

- **Company:** Syndax Pharmaceuticals
- **Med info phone:**781-419-1400
- **Med info email:**medinfo@syndax.com
- **Notes:** Expanded access NCT05918913 exists but only for patients under 40 kg, so it does not apply here. Ex-US managed-access program via World Orphan Drug Alliance for territories where Revuforj is not approved.

Payer / coverage: Approved on-label for KMT2A-translocation acute leukemia and NPM1-mutated AML. This patient has neither lesion (KMT2A amplification, NPM1-wild-type), so use would be off-label with the target essentially absent, making both payer coverage and clinical rationale weak.

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4. Tagraxofusp (SL-401, IL-3/diphtheria-toxin fusion)

Aliases: tagraxofusp, tagraxofusp-erzs, SL-401, Elzonris

Modality: other · **Regulatory:** FDA-approved (Elzonris, BPDCN, 2018); off-label for AML · **Guidelines:** NCCN-listed for BPDCN; off-guideline for AML · **Geographic scope:** US; approved drug available nationally, trial single-site (Stanford) · **Verified:** 2026-07-24

Tagraxofusp is FDA-approved for BPDCN, so a clinician could prescribe it off-label against the patient's CD123-positive blasts, though payers rarely cover off-label AML use without an appeal. The more structured route is the recruiting Stanford trial pairing it with low-intensity chemo in CD123+ R/R AML. Both hinge on the pending quantitative CD123 density and on organ-function data, since capillary-leak and hepatic toxicity are the load-bearing risks.

Next steps

1. Obtain the quantitative CD123 antigen-density flow result, which predicts benefit and is still pending
2. Get baseline organ-function labs (LFTs, albumin, cardiac) before any tagraxofusp exposure given capillary-leak risk
3. Email the Stanford trial contact wooin@stanford.edu (650-721-2443) to check eligibility and slot availability for NCT06561152
4. For an off-label prescription instead, call Stemline medical information at 609-613-2963 and Stemline ARC at 1-833-478-3654 for access/prior-auth support

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06561152	1/2	recruiting	likely	Woo In (Yustina) Cho; wooin@stanford.edu; 650-721-2443

Manufacturer / sponsor contact

- **Company:** Stemline Therapeutics (Menarini Group)
- **Med info phone:**609-613-2963
- **Product info:**https://www.elzonris.com/

- **Notes:** Adverse-event line 1-877-332-7961. Stemline ARC patient support / access 1-833-478-3654 (StemlineArc@pharmacord.com).

Payer / coverage: Approved and reimbursed on-label for BPDCN. Off-label AML use is not on the FDA label and would need prior authorization and a payer appeal; Stemline ARC (1-833-478-3654) handles access and assistance questions.

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5. Ziftomenib (menin inhibitor)

Aliases: ziftomenib, KO-539, Komzifti

Modality: small_molecule · **Regulatory:** FDA-approved (Komzifti, R/R NPM1-mutated AML, Nov 2025); off-label for this patient · **Geographic scope:** US; approved drug, no currently-enrolling matched trial · **Verified:** 2026-07-24

Ziftomenib was FDA-approved as Komzifti for NPM1-mutated R/R AML in November 2025, so off-label prescription is nominally available, but the patient is NPM1-wild-type and carries KMT2A amplification rather than a rearrangement, so the menin-inhibitor target is essentially absent. The COMET-001 trial is closed to enrollment. As with revumenib, this is on the list for completeness rather than as a realistic lever.

Next steps

1. Note the biomarker mismatch: neither an NPM1 mutation nor a KMT2A rearrangement is documented, so menin inhibition lacks a target
2. If relapse NGS ever shows a susceptible NPM1 mutation, call Kura medical information at 1-844-587-2662 (medinfo@kuraoncology.com) to discuss on-label use
3. Do not prioritise over the HLA-restricted and CD123 routes that match this patient's actual features

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT04067336	1/2	active not recruiting	no	—

Manufacturer / sponsor contact

- **Company:** Kura Oncology (with Kyowa Kirin)
- **Med info phone:**1-844-587-2662
- **Med info email:**medinfo@kuraoncology.com
- **Product info:**https://kuraoncology.com/komzifti/
- **Notes:** Kura medical information 1-844-KURAONC (587-2662) / medinfo@kuraoncology.com.

Payer / coverage: Approved on-label only for NPM1-mutated R/R AML. This patient is NPM1-wild-type with KMT2A amplification rather than a rearrangement, so off-label use has an absent target and weak coverage prospects.

Clinical trial (13)

6. BSB-1001 (HA-1-directed TCR-T)

Aliases: BSB-1001

Modality: other · **Regulatory:** investigational, no approval · **Geographic scope:** US only, 6 sites · **Verified:** 2026-07-24

BSB-1001 is a newer HA-1 TCR-T covering the same A*02:01-restricted mismatch, so it broadens the options if a TScan or Fred Hutch slot is unavailable. It is recruiting at six US sites. The eligibility gate is identical and still unresolved: HLA-A*02:01 plus recipient HA-1(H)-positive / donor HA-1-negative.

Next steps

1. Resolve the HLA-A*02:01 and HA-1 typing before contacting sites
2. Email BlueSphere's medical director at nkhan@bluespherebio.com (252-347-4938) to check slot availability at the nearest of the six sites
3. Treat this as a parallel option to TSC-100 and the Fred Hutch program so a slot can be found across whichever site opens first

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06704152	1/2	recruiting	unconfirmed	Nawazish Khan, MD (Medical Director, BlueSphere Bio); nkhan@bluespherebio.com; 252-347-4938

Manufacturer / sponsor contact

- **Company:** BlueSphere Bio
- **Med info phone:**252-347-4938
- **Med info email:**nkhan@bluespherebio.com

7. BSB-2002 (mutant-NPM1-directed TCR-T)

Aliases: BSB-2002

Modality: other · **Regulatory:** investigational, no approval · **Geographic scope:** US only, single site (St Louis) · **Verified:** 2026-07-24

BSB-2002 carries the same NPM1 target-absence problem as the Miltenyi program plus a disease-state mismatch, since it enrolls MRD-positive patients and this patient is in florid 85%-blast relapse and NPM1-wild-type. It is informational for the A*02:01 NPM1 TCR-T landscape, not an enrollment route here.

Next steps

1. Do not pursue: the patient is NPM1-wild-type and not in the MRD-positive population this trial enrolls
2. Revisit only if relapse NGS shows a susceptible NPM1 mutation type A/D/G/H and disease is brought to MRD

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT07566585	1	recruiting	no	Nawazish Khan, MD (Medical Director, BlueSphere Bio); nkhan@bluespherebio.com; 252-347-4938

Manufacturer / sponsor contact

- **Company:** BlueSphere Bio
- **Med info phone:**252-347-4938
- **Med info email:**nkhan@bluespherebio.com

8. CBX-250 (CG1/HLA-A*02:01 TCR-mimetic T-cell engager)

Aliases: CBX-250, CBX250, CROSSCHECK-001

Modality: bispecific_other · **Regulatory:** investigational, first-in-human · **Geographic scope:** US only, 11 sites · **Verified:** 2026-07-24

CBX-250 sits on the same HLA-A*02:01 axis as the HA-1/HA-2 and WT1 options, but as an off-the-shelf T-cell engager it skips the cell-manufacturing wait, which matters in a fast-moving 85%-blast relapse. It is recruiting at 11 US sites and allows prior transplant after 60 days. It stays a first-in-human phase 1 with no efficacy data and the usual cytokine-release risk, and the A*02:01 typing is still pending.

Next steps

1. Confirm HLA-A*02:01 status, the single gating biomarker for this engager
2. Email rachel.ghiraldi@crossbowtx.com (857-301-6432) to identify the nearest active site and confirm the 60-day post-transplant window is met
3. Weigh the faster off-the-shelf timeline against the lack of efficacy data when sequencing against the HA-1 cell products
4. Plan CRS monitoring given the T-cell-engager mechanism and high blast burden

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06994676	1	recruiting	unconfirmed	Rachel Ghiraldi; rachel.ghiraldi@crossbowtx.com; 857-301-6432

Manufacturer / sponsor contact

- **Company:** Crossbow Therapeutics
- **Med info phone:**857-301-6432
- **Med info email:**rachel.ghiraldi@crossbowtx.com

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9. CD123-CD33 compound CAR-T (cCAR)

Aliases: CD123-CD33 cCAR

Modality: CAR-T · **Regulatory:** investigational, no approval · **Geographic scope:** Listed sites in China; US-based sponsor; status unverified · **Verified:** 2026-07-24

The dual CD123/CD33 compound CAR-T matches the patient's antigen profile and the escape-hedging logic the profile calls for, but the ClinicalTrials.gov record carries an unknown (stale) recruitment status and the listed sites are in China. Whether the trial is still open is unverified, and there is no US site. Confirm status with the sponsor before treating this as reachable.

Next steps

1. Email the sponsor at kevin.pinz@icellgene.com (631-538-6218) to confirm whether the trial is still enrolling and whether any US site exists
2. Confirm a US/travel-accessible site is accepting the patient's geography before further workup
3. Quantify CD33 and CD123 antigen density to support dual-targeting rationale

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT04156256	early phase 1	unknown	unconfirmed	Kevin Pinz; kevin.pinz@icellgene.com; 6315386218

Manufacturer / sponsor contact

- **Company:** iCell Gene Therapeutics
- **Med info phone:**6315386218
- **Med info email:**kevin.pinz@icellgene.com
- **Notes:** US-based sponsor with listed trial sites in China; confirm whether any US site or an updated protocol is active.

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10. CLL-1/CD33/CD123-directed CAR-T

Aliases: CLL-1/CD33/CD123 CAR-T

Modality: CAR-T · **Regulatory:** investigational, no approval · **Geographic scope:** China only, single site (Shenzhen) · **Verified:** 2026-07-24

This multi-antigen CAR-T covers CD33 and CD123 and is recruiting, but only at a single academic site in

Shenzhen, China. For a US-based patient the barrier is geography and the logistics of overseas cell therapy, not biology. The profile does accept travel, so it stays on the list, but a China-only enrollment needs confirming before anything else.

Next steps

1. Confirm the Shenzhen site is accepting international patients and understand the travel and manufacturing logistics before workup
2. Email c@szgimi.org (+86 0755-86573763) to check eligibility with a prior sibling allo-HCT
3. Quantify CD33 and CD123 density to support the multi-antigen rationale
4. Weigh the practicality of overseas cell therapy against the recruiting US HA-1 and CBX-250 options

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT04010877	1/2	recruiting	unconfirmed	Lung-Ji Chang, Ph.D; c@szgimi.org; +86 0755-86573763

Manufacturer / sponsor contact

- **Company:** Shenzhen Geno-immune Medical Institute (academic)
- **Med info phone:**+86 0755-86573763
- **Med info email:**c@szgimi.org

11. FH-WT1-E50 (anti-WT1 TCR/CD8ab T cells) + azacitidine

Aliases: FH-WT1-E50, anti-WT1 TCR/CD8ab

Modality: other · **Regulatory:** investigational, no approval · **Geographic scope:** US only, single site (Seattle) · **Verified:** 2026-07-24

FH-WT1-E50 is the current WT1 A*02:01 TCR-T, but it enrolls MRD-positive AML and the patient is in overt high-burden relapse, so the disease state does not fit now. It becomes worth revisiting as consolidation once cytoreduction or a second transplant has driven disease down. A*02:01 and WT1 expression both need confirming.

Next steps

1. Hold as a consolidation option rather than a current-state fit given the 85%-blast burden
2. Confirm HLA-A*02:01 and quantify WT1 expression on the blasts in parallel with the other HLA workup
3. If disease is reduced to MRD after cytoreduction or HCT2, call the Fred Hutch intake line at 206-606-4668 to re-screen

Trial pathways

NCT	Phase	Status	Eligible	Central contact

NCT07645469	1	recruiting	no	Fred Hutch Immunotherapy Intake; immunotherapy@fredhutch.org; 206-606-4668

Manufacturer / sponsor contact

- **Company:** Fred Hutchinson Cancer Center (academic sponsor)
- **Med info phone:**206-606-4668
- **Med info email:**immunotherapy@fredhutch.org

12. HA-1-specific CD8+/CD4+ TCR-T (Bleakley/Fred Hutch platform)

Aliases: HA-1 TCR-T, HA-1 T TCR

Modality: other · **Regulatory:** investigational, no approval · **Geographic scope:** US only, single site (Seattle) · **Verified:** 2026-07-24

This is the closest published precedent for the patient's primary HA-1 handle and it is recruiting at Fred Hutch, but enrollment is single-site in Seattle and requires a fresh HLA-matched allo-HCT plus an A*02:01 restriction and a recipient HA-1(H)-positive / donor HA-1-negative mismatch. None of the gating typing is back yet, so eligibility cannot be established today. The intake line is the first call once the HLA and HA-1 genotypes are in hand.

Next steps

1. Retrieve the patient's HLA-A*02:01 allele status and genotype HA-1 in both the patient and the sibling donor before any outreach
2. Email immunotherapy@fredhutch.org (206-606-4668) to confirm the trial takes patients heading to a second allo-HCT and to reserve a screening slot
3. Confirm recipient-derived relapse by STR chimerism on sorted CD34+ blasts, which the protocol expects
4. Ask the site whether cell-manufacturing lead time can be aligned with the planned HCT2 timing

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT03326921	1	recruiting	unconfirmed	FHCC Immunotherapy Intake; immunotherapy@fredhutch.org; 206-606-4668

Manufacturer / sponsor contact

- **Company:** Fred Hutchinson Cancer Center (academic sponsor)
- **Med info phone:**206-606-4668
- **Med info email:**immunotherapy@fredhutch.org
- **Notes:** Academic program; the immunotherapy-intake line doubles as the sponsor contact. Senior investigator Marie Bleakley.

13. Iomab-B (131I-apamistamab) anti-CD45 radioimmunotherapy conditioning

Aliases: Iomab-B, 131I-apamistamab, iodine-131 apamistamab, BC8, anti-CD45 RIT, apamistamab

Modality: radioimmunotherapy · **Regulatory:** investigational, no approval (SIERRA phase 3 completed; not FDA-approved) · **Geographic scope:** US; go-forward trial not yet open, SIERRA closed · **Verified:** 2026-07-24

CD45-targeted radioimmunotherapy is the most directly relevant way to carry an 85%-blast marrow into a second transplant, and SIERRA showed it can do that in active R/R AML. The catch is access: SIERRA is closed to enrollment and the go-forward study is not yet recruiting, so there is a timing gap and the agent is not approved. SIERRA also enrolled patients going to a first transplant, so prior-HCT eligibility is the axis to verify for the go-forward protocol.

Next steps

1. Email the Actinium go-forward-trial contact mvusirikala@actiniumpharma.com (347-814-2268) to ask when NCT07157514 opens and whether a prior sibling allo-HCT is allowed
2. Ask directly whether any compassionate or single-patient access to apamistamab conditioning is possible during the enrollment gap
3. Confirm which sites will open and whether travel is feasible within the patient's disease tempo
4. Coordinate timing with the HCT2 plan, since this is a conditioning platform rather than a standalone therapy

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT07157514	2/3	not yet recruiting	unconfirmed	Madhuri Vusirikala, MD; mvusirikala@actiniumpharma.com; 347-814-2268
NCT02665065	3	active not recruiting	no	—

Manufacturer / sponsor contact

- **Company:** Actinium Pharmaceuticals
- **Med info phone:**347-814-2268
- **Med info email:**mvusirikala@actiniumpharma.com
- **Notes:** Actinium medical monitor listed as the go-forward trial contact. Iomab-B is not FDA-approved; ex-US commercialization agreement with Immedica (EMEA) is in place but the product is not yet approved.

Payer / coverage: Not approved, so access is trial-only. No standard reimbursement pathway exists for the conditioning agent outside a study.

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14. MB-dNPM1-TCR.1 (mutant-NPM1-directed TCR-T)

Aliases: MB-dNPM1-TCR.1, dNPM1-TCR.1

Modality: other · **Regulatory:** investigational, no approval · **Geographic scope:** EU only, single site (Leiden, Netherlands) · **Verified:** 2026-07-24

This option is off the table on biology: the TCR targets a mutant-NPM1 neoantigen and the patient is NPM1-wild-type, so the target is absent unless relapse NGS turns up an NPM1 mutation. It is also single-site in the Netherlands with a pending A*02:01 gate. Kept in the guide for landscape completeness rather than as a route.

Next steps

1. Do not pursue unless relapse NGS documents a susceptible NPM1 mutation, which the current panels do not show
2. If an NPM1 mutation ever appears, contact clinicaltrials.gov@miltenyi.com to check A*02:01 eligibility and whether a US site opens

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06424340	1/2	recruiting	no	Jorg Liebmann; clinicaltrials.gov@miltenyi.com; +49151-2034-4392

Manufacturer / sponsor contact

- **Company:** Miltenyi Biomedicine
- **Med info phone:**+49151-2034-4392
- **Med info email:**clinicaltrials.gov@miltenyi.com

15. Multiantigen leukemia-specific donor T cells (PRAME/WT1/Survivin/NY-ESO-1)

Aliases: TAA-T, tumor-associated antigen lymphocytes, TAA-specific CTL, mLST

Modality: other · **Regulatory:** investigational, no approval · **Geographic scope:** US only, 2 sites (Washington DC, Baltimore) · **Verified:** 2026-07-24

The multiantigen TAA-T product hits WT1 and PRAME without leaning on a single HLA-A*02:01 restriction, which makes it useful cover if the patient types A*02:01-negative. It is recruiting in the post-transplant setting the patient is heading toward, at Children's National and Johns Hopkins. The near-term to-do is confirming WT1 and PRAME expression on the blasts, since eligibility is built around antigen presence and the allo-HCT platform.

Next steps

1. Confirm WT1 and PRAME expression on the patient's blasts, which drives eligibility here
2. Email fhoq@childrensnational.org (202-476-3634) to confirm adult enrollment and the post-HCT timing the protocol expects
3. Note this avoids the A*02:01 gate, so keep it live even if HLA typing forecloses the restricted TCR options
4. Discuss whether the donor-derived product can be generated from the existing sibling donor

Trial pathways

NCT	Phase	Status	Eligible	Central contact

NCT	Phase	Status	Eligible	Central contact
NCT02203903	1	recruiting	unconfirmed	Fahmida Hoq, MBBS, MS; fhoq@childrensnational.org; 202-476-3634

Manufacturer / sponsor contact

- **Company:** Children's National Hospital / Catherine Bollard (academic sponsor)
- **Med info phone:**202-476-3634
- **Med info email:**fhoq@childrensnational.org

16. Rezatapopt (PC14586, Y220C-selective p53 reactivator)

Aliases: rezatapopt, PC14586

Modality: small_molecule · **Regulatory:** investigational, no approval · **Geographic scope:** US; myeloid trial single-site (MD Anderson) · **Verified:** 2026-07-24

Rezatapopt is the actionable TP53 handle only if the aberration is confirmed as Y220C, and the myeloid trial at MD Anderson is recruiting. The whole path is blocked behind two open questions: whether the patient is truly TP53-aberrant (2019 mutant vs a limited 2026-negative panel) and whether the variant is Y220C. The solid-tumor PYNNACLE study cannot enroll this patient. Resolving the TP53 variant identity is the gating step.

Next steps

1. Resolve TP53 status without collapsing the discrepancy: retrieve the 2019 variant identity and VAF, add p53 IHC and 17p FISH, and get variant-level NGS to establish whether it is Y220C
2. If Y220C is confirmed, email the MD Anderson contact cdinardo@mdanderson.org (713-794-1141) for NCT06616636 screening
3. Note the preclinical data show rezatapopt needs venetoclax to kill AML cells, and this patient already progressed through FLAG-IDA/venetoclax, so discuss expected depth with the site
4. Do not pursue if the variant is a non-Y220C TP53 aberration

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06616636	1b	recruiting	unconfirmed	Courtney DiNardo, MD; cdinardo@mdanderson.org; (713) 794-1141
NCT04585750	1/2	recruiting	no	PMV Pharma Clinical Study Information Center; clinicaltrials@pmvpharma.com; (609) 235-4038

Manufacturer / sponsor contact

- **Company:** PMV Pharmaceuticals
- **Med info phone:**(609) 235-4038

- **Med info email:**clinicaltrials@pmvpharma.com
- **Notes:** PMV Clinical Study Information Center handles trial and access inquiries.

Payer / coverage: Investigational; access is trial-only and there is no approval to reimburse against. Any compassionate request would go through PMV, but the drug is gated on a confirmed Y220C variant.

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17. TSC-100 (donor-derived HA-1-specific TCR-T)

Aliases: TSC-100, HA-1 TCR-T

Modality: other · **Regulatory:** investigational, no approval · **Geographic scope:** US only, 21 sites · **Verified:** 2026-07-24

TSC-100 is the commercial HA-1 TCR-T given after a fresh HLA-matched allo-HCT, and the ALLOHA trial is recruiting across 21 US sites, so geography is far less of a barrier than the single-site Fred Hutch program. Enrollment turns on HLA-A*02:01 plus a recipient HA-1(H)-positive / donor HA-1-negative mismatch, both untyped. The design lines up with the curative second-transplant plan.

Next steps

1. Resolve HLA-A*02:01 and HA-1 genotypes for patient and sibling donor first, since the whole arm hinges on the mismatch direction
2. Call TScan medical affairs at (857) 399-9500 or email medicalaffairs@tscan.com to find the nearest of the 21 sites and confirm slot availability
3. Confirm the protocol accepts a second transplant in a patient with a prior sibling allo-HCT
4. Line up the HCT2 timeline with the site so the TCR-T can follow conditioning

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT05473910	1	recruiting	unconfirmed	Tim White; medicalaffairs@tscan.com; (857) 399-9500

Manufacturer / sponsor contact

- **Company:** TScan Therapeutics
- **Med info phone:**(857) 399-9500
- **Med info email:**medicalaffairs@tscan.com
- **Notes:** Central-contact line is TScan medical affairs; use the same address for compassionate-access questions.

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18. TSC-101 (donor-derived HA-2-specific TCR-T)

Aliases: TSC-101, HA-2 TCR-T

Modality: other · **Regulatory:** investigational, no approval · **Geographic scope:** US only, 21 sites · **Verified:** 2026-07-24

TSC-101 is the HA-2 counterpart within the same ALLOHA trial, so it opens if the patient/donor pair turns out to mismatch at HA-2 instead of HA-1. It shares the HLA-A*02:01 gate and the fresh-transplant delivery, and it is recruiting at the same US sites. Which arm applies is decided by the HA-1/HA-2 genotyping that is still pending.

Next steps

1. Genotype both HA-1 and HA-2 in the patient and sibling donor, since the result selects between TSC-100 and TSC-101
2. Use the same TScan contact (857) 399-9500 / medicalaffairs@tscan.com to discuss whichever mismatch is found
3. Confirm prior-transplant eligibility and align with the HCT2 schedule

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT05473910	1	recruiting	unconfirmed	Tim White; medicalaffairs@tscan.com; (857) 399-9500

Manufacturer / sponsor contact

- **Company:** TScan Therapeutics
- **Med info phone:**(857) 399-9500
- **Med info email:**medicalaffairs@tscan.com

Unavailable (7)

19. BPX-701 (PRAME TCR-T with iCasp9 safety switch) + rimiducid

Aliases: BPX-701

Modality: other · **Regulatory:** investigational; program discontinued · **Geographic scope:** No open access (terminated) · **Verified:** 2026-07-24

Bellicum's BPX-701 was the PRAME-directed TCR-T the profile lists as a third A*02:01 handle, but the trial is terminated and the company has wound down, so there is no route. For a PRAME approach, the multiantigen TAA-T program (NCT02203903) is the open alternative.

Next steps

1. No action; the program is discontinued
2. Pursue PRAME targeting through the TAA-T multiantigen product (NCT02203903) instead

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT02743611	1/2	terminated	no	—

Manufacturer / sponsor contact

- **Company:** Bellicum Pharmaceuticals
- **Notes:** PRAME TCR-T discontinued; sponsor wound down operations. No successor protocol.

20. Eprenetapopt (APR-246, pan-p53 reactivator)

Aliases: eprenetapopt, APR-246, PRIMA-1MET

Modality: small_molecule · **Regulatory:** investigational; myeloid development largely halted after negative phase 3 · **Geographic scope:** No meaningful open AML route · **Verified:** 2026-07-24

Eprenetapopt is a pan-p53 reactivator, not Y220C-selective, and the profile already flags it as low-yield. Its pivotal TP53-mutant MDS phase 3 missed its endpoint and myeloid development has largely stopped, so there is no meaningful open trial route for this patient. If a p53-reactivation strategy is pursued, the Y220C-selective rezatapopt is the better-supported track, contingent on confirming a Y220C variant.

Next steps

1. No action recommended; the pan-reactivator track is low-yield and myeloid development has stalled
2. If TP53 Y220C is confirmed, pursue rezatapopt (NCT06616636) rather than eprenetapopt

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT04214860	1	completed	no	—

Manufacturer / sponsor contact

- **Company:** Aprea Therapeutics
- **Notes:** Myeloid development largely discontinued after the negative TP53-mutant MDS phase 3; no open enrolling AML route identified.

21. Flotetuzumab (MGD006, CD123xCD3 DART)

Aliases: flotetuzumab, MGD006, S80880

Modality: bispecific_other · **Regulatory:** investigational; program deprioritized · **Geographic scope:** No open access (terminated) · **Verified:** 2026-07-24

Flotetuzumab has some of the most relevant salvage data in the CD123 space, including a response signal specifically in TP53-abnormal AML, but MacroGenics deprioritized the program and the trial is terminated. There is no enrollment or expanded-access route. It remains useful as mechanism support for the CD123 bispecific idea, which is otherwise covered by CBX-250.

Next steps

1. No action; the program is deprioritized with no open route
2. For a CD123-directed T-cell-engager approach, pursue CBX-250 (NCT06994676) instead

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT02152956	1/2	terminated	no	—

Manufacturer / sponsor contact

- **Company:** MacroGenics
- **Notes:** CD123xCD3 DART program deprioritized; no open trial or expanded-access route identified.

22. NTLA-5001 (CRISPR-engineered WT1-directed TCR-T)

Aliases: NTLA-5001

Modality: other · **Regulatory:** investigational; program discontinued · **Geographic scope:** No open access (terminated) · **Verified:** 2026-07-24

Intellia terminated the NTLA-5001 WT1 TCR-T program in AML, so there is no enrollment route and no successor protocol. It appears here only to close the loop on the WT1 TCR-T pipeline the profile asked about.

Next steps

1. No action; the program is discontinued
2. For a WT1 TCR-T, follow FH-WT1-E50 (NCT07645469) instead

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT05066165	1/2	terminated	no	—

Manufacturer / sponsor contact

- **Company:** Intellia Therapeutics
- **Notes:** AML TCR-T program discontinued; no successor protocol.

23. Vibecotamab (XmAb14045, CD123xCD3 bispecific)

Aliases: vibecotamab, XmAb14045

Modality: bispecific_other · **Regulatory:** investigational; program discontinued · **Geographic scope:** No open access (terminated) · **Verified:** 2026-07-24

Vibecotamab was a second CD123xCD3 bispecific, studied in MRD-positive disease and now discontinued by Xencor. The disease-state mismatch and the program's closure both rule it out. It rounds out the CD123 bispecific sub-pipeline for completeness.

Next steps

1. No action; the program is discontinued

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT05285813	2	terminated	no	—

Manufacturer / sponsor contact

- **Company:** Xencor
- **Notes:** CD123xCD3 bispecific discontinued; no open route.

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24. WT1-TCRC4 EBV-specific CD8+ Tcm (Chapuis/Greenberg)

Aliases: WT1-TCRc4, WT1 TCR Tcm

Modality: other · **Regulatory:** investigational; study completed · **Geographic scope:** No open access (study completed) · **Verified:** 2026-07-24

The Chapuis WT1-TCRc4 work anchors the evidence that an A*02:01 WT1 TCR-T can prevent post-transplant AML relapse, but the trial is completed and the product has given way to FH-WT1-E50. There is no enrollment route to this specific construct. Follow the successor program instead.

Next steps

1. Treat as precedent only; the open WT1 TCR-T route is FH-WT1-E50 (NCT07645469)
2. Direct any WT1 TCR-T interest to the Fred Hutch immunotherapy intake line for the current protocol

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT01640301	1/2	completed	no	—

Manufacturer / sponsor contact

- **Company:** Fred Hutchinson Cancer Center (academic)

- **Med info phone:**206-606-4668
- **Med info email:**immunotherapy@fredhutch.org
- **Notes:** Legacy program; successor is FH-WT1-E50 (NCT07645469).

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25. WT1-siTCR gene-transduced lymphocytes (TBI-1301)

Aliases: WT1-siTCR, TBI-1301

Modality: other · **Regulatory:** investigational; early-phase study completed · **Geographic scope:** Japan academic only; no current open route · **Verified:** 2026-07-24

This A*24:02-restricted WT1-siTCR is the fallback lever the profile flags if the patient types A*02:01-negative, but the published work is a small completed Japanese academic cohort with no open successor trial in AML or MDS. It is not an accessible route today. It stays in the guide so its absence is not mistaken for an oversight on the A*24:02 branch.

Next steps

1. Prioritise resolving HLA-A*02:01 first; this branch only matters if that types negative
2. If A*02:01-negative and A*24:02-positive, ask academic transplant centers whether any current A*24:02 WT1 cellular program is open, since this specific TBI-1301 AML protocol is not

Trial pathways

NCT	Phase	Status	Eligible	Central contact
—	1	completed	unconfirmed	—

Manufacturer / sponsor contact

- **Company:** Mie University / Takara Bio
- **Notes:** Academic first-in-human work; no published open AML/MDS enrollment route. Would be relevant only on the A*24:02 branch if the patient types A*02:01-negative.
